

Cardiovascular Nursing Certification Blueprint

Target credential: ABCM CVRN-BC. Secondary alignment: ANCC CV-BC.

The course is built for the **ABCM CVRN-BC**. A second cardiovascular nursing credential, the ANCC CV-BC, is routinely confused with it, including in study guide marketing, and it tests the same subject matter through a completely different frame. Both blueprints are set out here from primary sources. Every competency in the course carries tags for both, so the ANCC frame stays available as a secondary readiness lens without a second build.

ABCM CVRN-BC

TARGET

BODY	American Board of Cardiovascular Medicine
FRAME	Clinical content categories
LEVELS	Level I non-acute, Level II acute
LENGTH	150 items Level I, 200 items Level II
ORGANIZED BY	Disease and modality

ANCC CV-BC

SECONDARY

BODY	American Nurses Credentialing Center
FRAME	Nursing process
LEVELS	Single credential
LENGTH	150 items, 125 scored, 25 pretest
ORGANIZED BY	What the nurse does, not what the patient has

Why the distinction is load bearing

A candidate sitting the ANCC exam who studies only from an ABCM-shaped review will know the pathophysiology and still lose points, because 60 percent of the ANCC score comes from planning, implementation, and patient education rather than from recognizing disease. A candidate sitting the ABCM exam who studies only from an ANCC-shaped review will have the reverse problem. The content overlaps almost completely. The weighting does not.

Same content, different centers of gravity

WHERE THE POINTS SIT

EMPHASIS	ABCM CVRN-BC	ANCC CV-BC
----------	--------------	------------

EMPHASIS	ABCM CVRN-BC	ANCC CV-BC
Recognizing and understanding disease	Dominant. Categories are named for conditions and modalities.	22 percent, inside Assessment and Diagnosis.
Choosing and carrying out interventions	Distributed inside each clinical category.	35 percent, its own category.
Evaluating response and adjusting	Distributed, largely implicit.	18 percent, its own category.
Teaching patients and communities	Minor, inside secondary prevention.	25 percent, its own category.

Prepared by Dr. Sharilyn Rennie · MedMasters Collaborative · August 13, 2026. Sources: ANCC Cardiac-Vascular Nursing Board Certification Test Content Outline, effective December 5, 2024, last updated November 1, 2024. ABCM published exam category list, abcmcertification.com/exams, retrieved August 13, 2026.

BLUEPRINT ONE

ABCM CVRN-BC

Clinical content categories, Level I and Level II

Level II is a superset of Level I. A Level I candidate is examined on seven categories and a Level II candidate on all fourteen, so a single course can serve both by gating the acute-care half behind the target level rather than building two courses.

LEVEL I, NON-ACUTE CARE, 150 ITEMS, WEIGHTS SUM TO 100 ACROSS SEVEN CATEGORIES

#	CATEGORY	WEIGHT	SOURCE
1	Bedside Assessment	10%	ESTIMATE
2	Basic ECG Essentials	14%	ESTIMATE
3	Coronary Artery Disease	22%	REPORTED
4	Hypertension	12%	ESTIMATE
5	Cardiomyopathy	9%	ESTIMATE
6	Heart Failure	20%	REPORTED
7	Non-Invasive and Interventional Cardiology	13%	ESTIMATE
Total		100%	

LEVEL II, ACUTE CARE, 200 ITEMS, ALL FOURTEEN CATEGORIES, WEIGHTS SUM TO 100

#	CATEGORY	WEIGHT	WHAT IT COVERS
1	Bedside Assessment	6%	History, precordial and vascular exam, escalation
2	Basic ECG Essentials	8%	Rate, rhythm, blocks, lethal rhythm response
3	Coronary Artery Disease	14%	ACS spectrum, biomarkers, reperfusion, prevention
4	Hypertension	6%	Measurement, pharmacology, urgency and emergency
5	Cardiomyopathy	5%	Types, obstruction physiology, SCD risk
6	Heart Failure	13%	Staging, GDMT, decompensation, self-care

#	CATEGORY	WEIGHT	WHAT IT COVERS
7	Non-Invasive and Interventional	7%	Test selection, cath and PCI, post-procedure, contrast
8	Heart Sounds and Valvular Disorders	5%	Auscultation, murmurs and maneuvers, valve lesions
9	12-Lead ECG Essentials	8%	Axis, blocks, localization, STEMI equivalents and mimics
10	Principles of Oxygenation	4%	Delivery and consumption, ABG, devices, PEEP effects
11	Hemodynamic Monitoring	7%	Lines and waveforms, derived values, shock profiles
12	Cardiac Assist Device Therapy	4%	Balloon pump timing, VAD, ECMO, emergencies
13	Pacemaker Interpretation	5%	Codes and modes, capture and sensing, malfunction, ICD
14	Cardiovascular Pharmacology	8%	Antithrombotics, antiarrhythmics, vasoactives, GDMT
Total		100%	

Outstanding request

ABCM publishes the category list but not a full percentage table. Coronary artery disease at 22 percent and heart failure at 20 percent are the Level I figures currently in hand and they anchor the rest. Every other weight above is an estimate, allocated by clinical load and objective count, and each carries a weightSource flag in the competency registry so the whole set can be replaced in one pass without touching content. ABCM provides a detailed study list on request by email. That request should go out before the item bank is locked, because weighting drives both how practice exam forms are assembled and how the weakness queue ranks what to study next.

BLUEPRINT TWO

ANCC CV-BC

Test content outline, effective December 5, 2024

TOTAL ITEMS

150

on the form

SCORED

125

score is based on these

PRETEST

25

unscored,
indistinguishable

CATEGORIES

4

by nursing process

SCORED ITEM DISTRIBUTION

#	CONTENT DOMAIN	ITEMS	PERCENT
I	Assessment and Diagnosis	27	22%
II	Planning and Implementation	44	35%
III	Evaluation and Modification	23	18%
IV	Patient and Community Education	31	25%
Total		125	100%

Subject areas within each domain

I. Assessment and Diagnosis · 22%

Knowledge. Anatomy and physiology. Pathophysiology. Nursing diagnosis identification.

Skill. Patient interview including history, chief complaint, and social determinants of health. Cardiac-vascular assessment techniques and tools such as Doppler and stroke scale. Data collection and interpretation covering diagnostic tests, laboratory results, and cardiac rhythms.

II. Planning and Implementation · 35%

Knowledge. Evidence-based practice guidelines including ACC and AHA guidelines and quality measures. Scope and standards of practice. Legal and ethical considerations including informed consent and advance directives. Invasive and non-invasive procedures. Surgeries including bypass and transplant. Pharmacologic therapies. Non-pharmacologic and complementary therapies. Risk-reduction measures including VTE prophylaxis and infection prevention.

Skill. Prioritization and care coordination including interdisciplinary teams and discharge

planning. Interventions including targeted temperature management and cardioversion.

III. Evaluation and Modification · 18%

Knowledge. Expected outcomes. Drug interactions, both drug to drug and drug to food.

Skill. Recognition and treatment of adverse reactions and events such as heparin-induced thrombocytopenia and hypotension. Recognition and treatment of urgent conditions such as pseudoaneurysm and hypertensive crisis. Recognition and treatment of emergent conditions such as pulmonary emboli and stroke.

IV. Patient and Community Education · 25%

Knowledge. Cardiac-vascular risk factors. Chronic disease management. Education topics covering procedures and medications. Self-management strategies such as daily weights and blood pressure logs. Community resources such as cardiac rehabilitation and anticoagulation clinics.

Skill. Individualized education planning and implementation addressing barriers and health literacy. Outpatient monitoring including remote telemetry and point-of-care testing. Health promotion including wellness counseling and health fairs.

Reproduced from the ANCC Cardiac-Vascular Nursing Board Certification Test Content Outline, effective December 5, 2024, last updated November 1, 2024. Examples in the outline are stated to be non-exhaustive and not ranked by importance.

The crosswalk

Secondary alignment to the ANCC nursing-process frame

The course is built on clinical content domains, which map one to one onto the ABCM categories. Every competency additionally carries an ANCC tag, so the dashboard can report a second readiness figure against the nursing-process frame using the same evidence. This is not a second course. It is a second reading of the same performance, and it is what lets the dashboard say "ready for CVRN-BC, thin on ANCC Domain IV" without any extra work from the learner.

COURSE DOMAIN TO ANCC DOMAIN

COURSE CONTENT DOMAIN	PRIMARY ANCC DOMAIN	ALSO FEEDS
D1 Bedside Assessment	I. Assessment and Diagnosis	IV. Education
D2 Basic ECG Essentials	I. Assessment and Diagnosis	III. Evaluation
D3 Coronary Artery Disease	II. Planning and Implementation	I, III, IV
D4 Hypertension	IV. Patient and Community Education	II, III
D5 Cardiomyopathy	I. Assessment and Diagnosis	II, IV
D6 Heart Failure	II. Planning and Implementation	III, IV
D7 Non-Invasive and Interventional Cardiology	II. Planning and Implementation	III
D8 Heart Sounds and Valvular Disorders	I. Assessment and Diagnosis	II
D9 12-Lead ECG Essentials	I. Assessment and Diagnosis	III
D10 Principles of Oxygenation	II. Planning and Implementation	III
D11 Hemodynamic Monitoring	III. Evaluation and Modification	I, II
D12 Cardiac Assist Device Therapy	II. Planning and Implementation	III
D13 Pacemaker Interpretation	III. Evaluation and Modification	II, IV
D14 Cardiovascular Pharmacology	II. Planning and Implementation	III, IV

The gap this exposes

Eight of seventy-eight competencies map to ANCC Domain IV, which carries 25 percent of that exam. That is fine for the CVRN-BC target, where patient teaching sits inside secondary prevention rather than standing alone. It means the ANCC readiness figure will read low until a dedicated education and health promotion strand is added, covering health literacy, teaching barriers, community resources, outpatient monitoring, and wellness counseling. Left as is, the secondary figure is honest about a real gap rather than hiding it.

The failure pattern

What experienced cardiac nurses get wrong on these exams

The most common way a strong bedside nurse fails is not a knowledge deficit in the usual sense. It is over-reliance on day-to-day cardiac experience in two areas that reward something practice does not routinely require. The pattern is named in ANCC terms below because that is where the weights make it starkest, at 60 percent of that score. The second of the two carries directly into the CVRN-BC target, where guideline reasoning is distributed through every clinical category rather than isolated in one.

Blind spot one · Patient and Community Education ANCC 25%

The second largest block, and the one inpatient-focused candidates encounter least. It tests outpatient cardiac rehabilitation, anticoagulation clinic teaching, daily weight and blood pressure self-monitoring, health literacy, and community-level risk factor reduction. A nurse who spends every shift on a cardiac floor may go years without touching most of this, because it lives downstream of discharge.

What the course does about it. Education is built as its own strand with its own dashboard tile, not folded into disease domains as an afterthought. It gets scored practice at the same depth as ECG. A learner whose readiness is high everywhere else and low here sees exactly that, which is the whole point of separating the tile.

Blind spot two · Guideline pathways ANCC 35% CARRIES INTO CVRN-BC

The explicit ACC and AHA guideline pathways for STEMI and NSTEMI, heart failure, hypertension, and atrial fibrillation anticoagulation. Working nurses apply these constantly without ever having studied them as pathways. They know what happens on their unit. The exam asks what the guideline says and why, including the decision points their institution has already made for them in a protocol. On the CVRN-BC this is not a separate category, it is threaded through coronary artery disease and heart failure, which together carry 42 percent of Level I.

What the course does about it. Guideline-pathway items are written to surface the decision rule rather than the local habit: why this threshold, what changes the answer, what the alternative pathway is when the first is contraindicated. Where a protocol normally hides a choice, the item exposes the choice.

How this changes the build

IMPLICATION	CHANGE
Experience inflates self-assessment	The gap finder does not trust self-rating alone. Pass one collects it, pass two tests the heaviest domains regardless of how the learner rated them, because self-assessment is reliable about known unknowns and unreliable about everything else.
Education is under-practiced, not under-known	Education competencies get scored items and spaced review, not reading pages. The strand carries the same DOK ladder as the clinical domains.
Guideline reasoning is invisible in practice	Planning and implementation items are written at DOK 3, asking for the decision and its justification, rather than DOK 1 recall of a protocol step.
Confidence is the risk factor	Confident and incorrect responses are flagged separately and given the highest review priority. This is the population most likely to generate them.

This page reflects the instructional design decision to weight the course against the observed failure pattern rather than against subject familiarity. Prepared by Dr. Sharilyn Rennie.

Competency structure

How the blueprint becomes a course

DOMAINS

14

7 at Level I

COMPETENCIES

78

37 at Level I

OBJECTIVES

311

147 at Level I

ECG STRANDS

5

parallel spine

Three tiers

TIER	ID	WHAT IT IS	WHAT IT DRIVES
Domain	D6	A blueprint category	Dashboard tile, exam form proportions
Competency	D6.C4	A study unit, one session	What a learner navigates and schedules
Objective	D6.C4.O2	One observable behavior	What each item is tagged to, and the level the dashboard diagnoses

The ECG spine

ECG runs as a parallel strand rather than three scattered categories, because interpretation is a compounding skill and it is where candidates lose the most points. The spine does not duplicate objectives, it re-indexes them, so one mastery score is read through two lenses.

STRAND	NAME	SOURCED FROM	CROSS-LINKS INTO
E1	Waveform literacy	D2	D14
E2	Rhythm diagnosis under time pressure	D2	D5, D6, D12
E3	12-lead ischemia and localization	D9	D3, D7
E4	Paced and device tracings	D13	D12
E5	Tracing to patient integration	D9, D2	D1, D8, D10, D11

Depth and mastery

Four depth levels

DOK 1 recall. DOK 2 application, the level most certification items sit at. DOK 3 analysis, deciding and justifying with a patient attached. DOK 4 synthesis across changing data. A competency is not marked mastered until its DOK 3 objectives clear, which is what keeps the dashboard honest about the difference between recognizing a rhythm and managing the patient in it.

Four mastery states

Mastered, Fragile, Gap, and Untested, driven by confidence-scored responses. Answering incorrectly while confident carries its own flag and the highest review priority, because a learner who knows they do not know something will study it, and a learner who is certain of something wrong will walk past it every time.

Decisions still open

1. **Settled.** The course targets the ABCM CVRN-BC. ANCC CV-BC remains a tagged secondary lens on the dashboard, and adding the education strand would make it a full second target if that is ever wanted.
2. **ABCM category weights.** Request the detailed study list by email. Two of seven Level I weights are confirmed; the rest are flagged estimates and swap out in one pass.
3. **Items per objective.** Four gives diagnostic resolution. Six gives a practice exam pool as well. At 311 objectives this is the number that sets the authoring workload.
4. **Practice exam pool depth.** Forms run either as a 30-item quick check or at full length, 150 items for Level I and 200 for Level II. Ten full-length Level II forms would require 2,000 reserved items, which is not the realistic target. A pool of roughly 600 reserved items supports ten honest quick checks plus two full-length forms. Sixty are authored.
5. **Whether DOK 4 uses drawing-based synthesis** the way the anatomy courses do, and what the capture mechanism is in a self-paced online setting.

Prepared by Dr. Sharilyn Rennie · MedMasters Collaborative · ECG & CVRN Review Course · Competency framework v0.1 · August 13, 2026